Supplementary table 1: Health economic outcomes

Author Perspective (currency year)	Cost categories	Δ Costs	Health-related outcome	ICER[a]	CEA[b] P (WTP = 0); WTP (*p* = 0.5); WTP (*p* = 0.95)	QALY	ICUR[a]	CUA[b] P (WTP = 0; £20,000; £30,000)	CBA	Results[c]
Boiler, 2014 Societal (2009, €)	- Direct costs: medical, non-medical, intervention - Indirect costs: productivity losses Source: TiC-P	€1,471	Symptom severity (CES-D < 16, 5 score change)**[d]	€9,807 NE	22%; >€100,000	N.A.	N.A.	N.A.	N.A.	Unguided iPPI vs. WLC: ICER: £11,226 per symptom severity iPPI generated higher effects at higher costs.
Buntrock, 2017 Societal, Healthcare (2013, €)	- Direct costs: healthcare, patient costs (e.g., traveling, opportunity), medication, intervention - Indirect costs: productivity losses Source: TiC-P	Societal: €143 Healthcare: €136	Depression free years (DFY)**[e]	Societal: €1,117 NE Healthcare: €1,125 NE	Societal: 38%; €2,000[f] €9,680 Healthcare: 17%; €3,920[f]	EQ-5D-3L	Societal: €13,400 NE Healthcare: €13,500 NE	Societal: 38%[f], ≥60%[f] Healthcare: N.R., ≥ 64%	N.A.	Guided iCBT vs. TAU⁺: ICER: £1,170 (societal), £1,178 (healthcare)/depression-free year ICUR: £14,034 (societal), £14,139 (healthcare)/QALY gained iPST/BA resulted in greater effects and more QALY gained at higher costs.
Gerhards, 2010 Societal (2007, €)	- Direct costs: medical costs (healthcare sector), non-medical costs (travel, use of iCBT) intervention - Indirect costs: productivity losses Source: PRODISQ, patient reported healthcare use	iCBT vs. TAU: €-711 iCBT+TAU vs. TAU: €738 iCBT vs. iCBT+TAU: €-1,449	Symptom severity (BDI-II): reliable change index	N.R.	iCBT:70[f] TAU:13[f] iCBT + TAU⁺: 18[f], outperforms others with increased WTP (max. 80%)	EQ-5D	N.A.	iCBT:65%[f], 57%[f], 55%[f] TAU: 25%[f], 30%[f], 33%[f] iCBT & TAU: 10%[f], 12%[f], 14%[f]	N.A.	Unguided iCBT vs. unguided iCBT + TAU⁺ vs. TAU[Nb] Costs were lowest for the iCBT. There were no significant group differences in effect or QALY compared to iCBT plus TAU+ and TAU only. CUA and CEA tend to be in favor of iCBT.
Phillips, 2014 N.R. (N.R.., £)	- Direct costs: hospital, community, and healthcare services - Indirect costs: lost workdays (total absence) Source: CSSRI, self-assessed absence from work	£-35	N.A.	N.A.	N.A.	EQ-5D	N.R., SW	N.R.	N.A.	Unguided iCBT vs. AC: iCBT is less costly but generates lower QALYs.

Author	Cost categories	Δ Costs	Outcome	ICER	CEA	QALY	ICUR	CUA	CBA	Results
Titov, 2015 Healthcare, (N.R., AU$)	- Direct costs: healthcare resource use (admissions, consultations, medication use), intervention (therapist´s time) Source: resource use, health-care visits	$52	N.A.	N.A.	N.A.	EQ-5D-5L*[d]	AU$4,392 NE	8%[e], 95%[e], 95%[e]	N.A.	Guided iCBT vs. WLC: ICUR: £2,466/QALY gained iCBT resulted more QALY at higher costs.
Van Luenen, 2019 Societal, Healthcare (2017, €)	- Direct costs: intervention, healthcare - Indirect costs: productivity losses Source: TiC-P	Societal: €-731 Healthcare: €11	N.A.	N.A.	N.A.	SF-6D	Societal: N.R., SE Healthcare: N.R., NE	Societal: 95%[f],96%[f], 98%[f] Healthcare: 48%[f], 88%[f], 94%[f]	N.A.	Guided iCBT vs. AC + WLC: iCBT generated less (societal) and higher (healthcare) costs/QALY gained.
Brabyn, 2016 Healthcare (2012/13, £)	- Direct costs: service use, medication, intervention (only telephone support) Source: CSSRI	£591	N.A.	N.A.	N.A.	EQ-5D-5L	N.R., SE	50%, 55%, 55%	N.A.	Guided iCBT vs. unguided iCBT: Guided iCBT resulted in more QALYs gained at lower costs.
Geraedts 2015 Societal, Employer (2012, €)	- Direct costs: medical/non-medical costs, occupational health, intervention - Indirect costs: productivity losses Source: TiC-P, WHO-HPQ	Societal: €-714 Employer: €-508	CES-D, clinically significant change	Societal: €-6,645 SE Employer: €-4,664 SE	Societal: 62%, N.A., €44,000 Employer: 55%, N.A., €115,000	EQ-5D-3 L	Societal: €532,959 SW Employer: €382,354 SW	Societal: N.R.; max. 62% Employer: N.R.; max. 55%	NB: €508 BCR: €2.8	Guided iPST vs. TAU[Nb]: ICER: £-6,486 (societal), £-4,552 (employer)/significant change ICUR: £520,203 (societal), £373,203 (employer)/QALY gained iPST dominated regarding clinically significant change but resulted in less QALYs gained at lower costs.
Hollinghurst, 2010 Healthcare (inferred societal) (2007, £)	- Direct costs: NHS service use, personal expenditures, intervention (no costs for IMI/website) - Indirect costs: productivity (workdays lost) Source: resource use diary	£469	BDI "Recovery" (BDI <10)	£3,528 NE	N.R.	EQ-5D	£17,173 NE	0%[f]; 56%[f], 75%[f]	N.A.	Guided iCBT vs. WLC[Nb]: ICER: £4,686/recovered person ICUR: £22,810/ QALY gained iCBT resulted in greater effects and more QALY gained at higher costs.

Author	Cost categories	Δ Costs	Outcome	ICER	CEA	QALY	ICUR	CUA	CBA	Results
Klein, 2018 Societal, (2014, €)	- Direct costs: intervention, healthcare, participant - Indirect costs: productivity losses Source: TiC-P	€1008	Depression-free days	N.R., NE	40%, €50[f], max 65%	EQ-5D-3L	N.R., NE	18%, max 40%	N.A.	iPCT +TAU vs. TAU: iPCT resulted in greater effects and more QALY gained at higher cost.
Littlewood, 2015 Healthcare (2011/12, £)	- Direct costs: primary care, hospital, other community services, medication, intervention Source: GP medical records	iCBT1 vs. uGPC: £104 iCBT2 vs. uGPC: £-106	N.A.	N.A.	N.A.	EQ-5D	iCBT1 vs. TAU: £6,933 SW iCBT2 vs. TAU: dominated NW	iCBT1: 94%[f]; 42%[f] 38%[f] iCBT2: 0%[f], 0.04%[f]; 4%[f] uGPC: 6%[f]; 55%, 58%[f]	N.A.	Unguided iCBT 1 vs. uGPC [Nb]: ICUR: £7,838/QALY gained iCBT1 resulted in lower costs and less QALY gained. Unguided iCBT 2 vs. uGPC: iCBT2 resulted in higher costs and less QALY gained. uGPC alone compared with iCBT was most likely to be cost-effective.
Nobis, 2018 Societal (2013, €)	- Direct costs: healthcare services, medication, travel expenses, intervention - Indirect costs: domestic help, productivity losses Source: TiC-P	€97	Treatment response (CES-D): Reliable change Index and ≤ 23	€233 NE	48%; €250[f] €4,800[f]	EQ-5D-3L	€10,708 NE	46%; N.R., €14,000 = 51%	N.A.	Guided iCBT (1) vs. TAU+: ICER: £244/treatment response ICUR: £11,215/QALY gained iCBT resulted in greater effects and more QALY gained at higher cost
Romero-Sanchiz, 2017 Societal (2014, €)	- Direct costs: healthcare sector (GP, hospital stays), medication -Indirect costs: absenteeism Source: CSSRI	iCBT1 vs. TAU[+]: €-409 iCBT2 vs. TAU[+]: €41	BDI-II, point improve-ment*[g]	iCBT1 €-98 SE iCBT2 €10 NE	N.R.	EQ-5D-3 L*[g]	iCBT1: €-5,160 SE iCBT2: €497 NE	N.R.	N.A.	Unguided iCBT1 vs. TAU[+]: ICER: £-99/point improvement ICUR: £-5,201 per QALY iCBT1 dominated TAU[+] Guided iCBT2 vs. TAU[+]: ICER: £10/point improvement ICUR: £501/QALY gained iCBT2 resulted in greater effects and more QALY gained at higher cost.

Author	Cost categories	Δ Costs	Outcome	ICER	CEA	QALY	ICUR	CUA	CBA	Results
Warmerdam, 2010 Societal (2007, €)	- Direct costs: medical/non-medical costs (e.g., traveling, parking), intervention -Indirect costs: domestic help, productivity losses Source: TiC-P	iCBT vs. WLC: €256 iPST vs. WLC: €147 iCBT vs. iPST: €109	Clinically significant change (CES-D < 16)	iCBT vs. WLC: €1,817 NE iPST vs. WLC: €1,248 NE iCBT vs. iPST: €-36, NW	iCBT vs. WLC: 30%, €2,500[f], €25.000[f] iPST vs. WLC: 38%, €2,000[f], €35.000[f] iCBT vs. iPST N.R.	EQ-5D	iCBT vs. WLC: €22,609 NE iPST vs. WLC: €11,523 NE iCBT vs. iPST: N.R.	iCBT vs. WLC: 28%; 44%[f], 55%[f] iPST vs. WLC: 38%; 52%[f], 63%[f] iCBT vs. iPST: N.R.	N.A.	Guided iCBT vs. WLC: ICER: £2,147/clinical change ICUR: £26,713/QALY gained Guided iPST vs. WLC: ICER: £1,475/clinical change ICUR: £13,615/QALY gained iCBT vs. iPST: ICER: £43/clinical change iCBT and iPST resulted in greater effects and more QALY gained at higher costs compared to WLC.
Yan, 2019 Healthcare (2017, $)	-Direct costs: healthcare (physician, outpatient, inpatient services e.g., salaries, drugs, medical supplies) Source: Health administrative databases	SCP vs.SC: -$155 SCP vs. TAU: -$449 SCP vs. iCBT: -$161	N.A.	N.A.	N.A.	EQ-5D-5L	N.R.	SC: 26%[f], 27%[f], 28%[f] TAU: 18%[f], 11%[f], 7%[f] iCBT: 27%[f], 29%[f],30%[f] SCP: 29%[f], 33%[f], 34%[f]	N.A.	SCP vs. SC, TAU, unguided iCBT[Nb]: QALYs were highest in SCP, followed by SC, CBT, and TAU. SCP had a higher probability for cost-effectiveness than the three alternatives.
Bergström, 2010 N. R., (N.R., €)	-Direct cost: therapist´s and psychiatrist´s time Source: self-assessed therapist time	€-239	Responder ≥ 40% reduction on PDSS	N.R. SE	75%; N.R.	N.A.	N.A.	N.A.	N.A.	Guided iCBT vs. gCBT: iCBT dominates gCBT, generating more effect (responder) at less costs.
Dear, 2015 Healthcare (2013, AU$)	-Direct costs: primary, secondary care, therapist or supervisor costs, medication, internet access, computer, telephone use Source: patients' healthcare resource use	$92	N.A.	N.A.	N.A.	EQ-5D-5L*[h]	$8,806 NE	0%[f], 93%[f], 96%[f]	N.A.	Guided iCBT vs. WLC: ICUR: £4,825/QALY gained iCBT resulted in more QALYs at higher costs.

Author	Cost categories	Δ Costs	Outcome	ICER	CEA	QALY	ICUR	CUA	CBA	Results
Nordgren, 2014 N.R. (N.R., US$)	- Direct cost: healthcare, participant medication, intervention (therapists) - Indirect costs: productivity losses Source: TiC-P	CEA $-616 CUA $-474	CORE-OM defined as responder **[h]	$-1,824 SE	95%; N.A.	EQ-5D, N.R.	$-7,523, SE	90%, > 95%	N.A.	Guided iCBT vs. AC: ICERs: £-1,254/responder ICUR: £-5,173/QALY gained iCBT generated less costs and more effects and QALY gained.
Hedman, 2013 Societal, (2010, £),	-Direct costs: intervention (only therapists), healthcare, participant -Indirect costs: productivity losses Source: TiC-P	£-784	HAI, no diagnostic criteria for severe health anxiety**[i]	£-1,244 SE	64%; £4.800[f]	EQ-5D**[i]	£−6,533 SE	67%, 91%[f]	N.A.	Guided iCBT vs AC: ICER £-1,512/person without diagnosis on HAI ICUR; £-7,940/QALY gained iCBT generated less costs per effect and QALY gained.
Hedman, 2016 Societal, (2013, US$)	-Direct cost: intervention (only therapists), healthcare, participant costs - Indirect costs: productivity losses Source: TiC-P	$310	HAI, responder clinically significant improvement*[i]	$2,214 NE	9%; $2,300; $20,000[f]	EQ-5D, N.R.	$10,000 NE	N.R.	N.A.	Unguided iCBT vs. unguided iMA[Nb]: ICER: £1,633/responder ICUR £7,376/QALY gained iCBT resulted in higher effect and more QALYs gained at higher costs.
Hedman, 2011 Societal, (2009, US$)	-Direct costs: intervention (only therapists), healthcare, participant costs -Indirect costs: productivity losses Source: TiC-P	$-1335	LSAS < 43.3, responder	$-7,046 SE	81%; $70,000[f]	EQ-5D	$-17, 823 SE	81%, 82%[f],79%[f]	N.A.	Guided iCBT vs. gCBT: ICER: £-5,729/responder ICUR £-14,491/QALY gained iCBT generated more effect and QALY gained at less costs.
Hedman, 2014 Societal, (N.R., US$)	- Direct costs: intervention (only therapists), healthcare, participant costs - Indirect costs: productivity losses Source: TiC-P	$-808	LSAS < 43.3, responder	$10,100 SW	62%, max. 62%	EQ-5D	-$7,345 SE	Max. 62%, 39%[f], 34%[f]	N.A.	Guided iCBT vs. gCBT: ICER: £11,749/responder iCBT resulted in less effect at lower costs. ICUR: £-8,544/QALY gained iCBT generated less costs per QALY gained.

Author	Cost categories	Δ Costs	Outcome	ICER	CEA	QALY	ICUR	CUA	CBA	Results
Alaoui, 2017 Healthcare (2017, €)	- Direct costs: hospital space, IT usage, security, management, therapists Source: TDABC	€-343	N.A.	N.A.	N.A.	EQ-5D	N.R.	100%[f], 68%[f], 67%[f]	N.A.	Guided iCBT vs. gCBT: Both treatments were equally efficacious but iCBT generated less costs.
Powell, 2020 Societal, Healthcare, (2016/17, £)	- Direct costs: healthcare costs, intervention - Indirect costs: workdays lost Source: PSSRU, UK	Social: £-65 Healthcare: £-63	N.A.	N.A.	N.A.	SF-6D	Social: N.R SE Healthcare: N.R SE	N.R.	N.A.	Unguided iCBT vs. WLC: iCBT resulted in less costs per QALY gained.
Andersson, 2015a Societal Healthcare (2013, US$)	- Direct costs: therapist, healthcare, participant costs - Indirect costs: productivity losses Source: TiC-P	Societal $503 Healthcare $336	Y-BOCS, additional remission**[j]	Societal $931 NE Healthcare $672 NE	Societal 15%[f], $ 1100, $2600 = 90% Healthcare 0%[f], $700[f], $900[f]	EQ-5D-5L**[k]	Societal $7,186 NE Healthcare $4,800 NE	Societal 15%[f]; >95%[f] Healthcare 0%[f]; >90%[f]	N.A.	Guided iCBT vs. AC[Nb]: ICER: £687 (societal), £496 (healthcare)/remission ICUR: £5,301 (societal), £3,541 (healthcare)/QALY gained iCBT resulted in greater effects and more QALYs at higher costs.
Andersson, 2015b Societal, Healthcare (2013, US$)	- Direct costs: therapists, healthcare, participant - Indirect costs: productivity losses Source: TiC-P	Societal: $338 Healthcare: $242	Y-BOCS, relapse avoided	Societal: $1,489 NE Healthcare: $1,066 NE	Societal: 18%[f], $1500[f], $7000[e] Healthcare 0%, $900[f], $4250[f]	N.A.	N.A.	N.A.	N.A.	iCBT booster session vs. TAU[Nb]: ICER: £1,098 (societal), £786 (healthcare)/relapse avoided The additional booster session generated higher effects at higher costs.
Lenhard, 2016 Societal, Healthcare (2016, US$)	- Direct costs: healthcare use, supportive resources, drugs, intervention - Indirect costs: school absence, productivity losses Source: TiC-P	Societal: $-145 Healthcare: $21	Treatment responder >34% on CY-BOCS **[l]	Societal: N.R., SE Healthcare: $78 NE	Societal: 59.4%; N.R. Healthcare: 48%[f]; $200[f], $3000[f]	EQ-5D-Y	Societal: N.R. SW Healthcare: N.R.	N.R.	N.A.	Guided iCBT vs. WLC[Nb]: Societal: iCBT generated less costs per treatment responder. iCBT generated slightly less QALY at lower cost. Healthcare: ICER: £57/ treatment responder iCBT produced higher effects at higher costs.

Author	Cost categories	Δ Costs	Outcome	ICER	CEA	QALY	ICUR	CUA	CBA	Results
Lovell, 2017 Societal, Healthcare (2013/14, £)	- Direct costs: hospital and community-based health- and social-care services, medication, out-of-pocket expenses and savings, intervention - Indirect costs: productivity losses Source: AD-SUS, WHO´s HPQ	Healthcare iCBT vs. WLC: £138 SH vs. WLC: £364 iCBT vs. SH: £-226 Societal iCBT vs. WLC: £200 SH vs. WLC: £310 iCBT vs. SH: £-109	N.A.	N.A.	N.A.	EQ-5D-3L QALY	Healthcare iCBT vs. WLC: £32,857 NE SH vs. WLC: £55,152 NE iCBT vs. SH: £94,167 SW Societal iCBT vs.WLC: £48,095 NE SH vs. WLC: £46,970 NE iCBT vs. SH: £45,417 SW	Healthcare iCBT vs. WLC: 17%[f], 42%[e], 52%[f] SH vs. WLC: 0%[e], 0%[f], 8%[f] iCBT vs. SH: 95%[f], 88%[f], 85%[f] Societal iCBT vs. WLC: 15%[f], 28%[f], 35%[f] SH vs. WLC: 0%[f], 5%[f], 15%[f] iCBT vs. SH: 72%[f], 65%[f], 60%[f]	N.A.	Guided iCBT & Self-help vs. WLC[Nb]: Healthcare: ICUR: iCBT (£35,699), SH (£59,923)/QALY gained Societal: ICUR: iCBT (£52,255), SH (£51,033)/QALY gained iCBT and SH gained more QALY at higher costs. iCBT vs. Self-help: ICUR: £102,312 (healthcare), £49,346 (societal)/QALY gained iCBT had less costs and slightly less QALY gained.
Röhr, 2021 Healthcare (2019, €)	- Direct costs: healthcare costs, intervention, Source: CSSRI	€-100	N.A.	N.A.	N.A.	EQ-5D-5L	N.R.	81%; 38% 27%	N.A.	Unguided iCBT vs. TAU[+Nb]: iCBT generated lower costs per QALY gained.
De Bruin, 2016 Societal (2014, €)	- Direct costs: doctor visits, medication use, traveling expenses - Indirect costs: informal care, parents´ loss of (non) paid work, tutoring of adolescents Source: cost diaries (parents)	Societal: €-406 Healthcare: €147	Subjective sleep efficiency, Recovery (SE ≥ 85%)	€12,572 SW	95%, €12,000	EQ-5D	N.R. SE	95%, 70%[f], N.R.	N.A.	Guided iCBT vs. gCBT: ICER: £13,153 /recovered person iCBT produced less effect at less costs. iCBT generated slightly more QALY at less costs.

Author	Cost categories	Δ Costs	Outcome	ICER	CEA	QALY	ICUR	CUA	CBA	Results
Thiart, 2016 Buntrock, 2021 Employer, Societal, Healthcare (2013, €)	- Direct costs: healthcare intervention - Indirect costs: productivity losses Source: TiC-P	Employer: €-418 Societal: €-991 Healthcare: €203	Positive treatment response (ISI)*[k] Symptom-free status (ISI)*[m]	Employer: €-1,512 SE Societal: N.R. SE Healthcare: €650	Employer: 87%, N.A., €761 Societal: 94%, N.A., €1000 Healthcare: 6%, €500, €1500	SF-6D*[m]	Societal: N.R. SE Healthcare: €11,285	Societal: 94%, 98%, 99% Healthcare: 4%, 95%, 99%	NB: €417 BCR: €3.1	Guided iCBT vs. WLC: Employer: ICER: £-1,583/responder iCBT dominated WLC and produced a net benefit of £437 and a BCR of £3.2. Societal: iCBT dominated WLC generating more effect and QALY at less costs. Healthcare: ICER: £681/symptom-free status ICER: £11,819/QALY gained iCBT generated more effect and QALY at higher costs.
Ebert, 2018 Kählke, 2019 Employer Societal, (2013, €)	- Direct costs: health service uptake, patients' out-of-pocket costs, intervention costs - Indirect cost: productivity losses Source: TiC-P	Employer: €-189 Societal: €-380	Symptom-free status by Jacobson and Truax**[d]	Employer: €-521 SE Societal: €-1,063 SE	Employer: 67%, N.A., €1.500[f] Societal: 70%, N.A., €2.500[f]	SF-6D	Societal: N.R. SE	Societal: 71%[f], 76%[f],79%[f]	NB €181 BCR €1.6	Guided iMA vs. WLC: ICER: £-546 (employer), £-1,113 (societal)/symptom-free person iMA generated higher effects and QALY gained at lower costs, and a net benefit of £190 and a BCR of 1.06.
Lindsäter, 2019 Societal, Healthcare (2016, US$)	- Direct cost: healthcare, intervention -Indirect costs: productivity losses Source: TiC-P	Societal: $-77.24 Healthcare: $171.12	Remission rate on PSS (reliable change, post-rating PSS<31)**[d]	Societal: $-158 SE Healthcare: $349 NE	Societal: 60%, N.A., $1000 Healthcare: 12%[f], $400[f], $1000[f]	EQ-5D-3L	Societal: $-17,963 SE	Societal: 60%, 71%[f], 75%[f]	N.A.	Guided iCBT vs. WLC: Societal: ICER: £-115/responder ICUR: $-13,109/QALY gained iCBT generated more effect and QALY at less costs. Healthcare: ICER: £-255 iCBT generated higher effects and costs.

Author	Cost categories	Δ Costs	Outcome	ICER	CEA	QALY	ICUR	CUA	CBA	Results
Van Spijker, 2012 Societal (2009, €)	- Direct cost: health service uptake, patients' out-of-pocket costs, intervention, medication - Indirect cost: productivity losses Source: TiC-P	€-5039 (annualised costs)	Treatment response (improvement of 6.48 on BSS)*[n]	€-34,727 SE	91.5%, N.A., €18,000[f]	N.A.	N.A.	N.A.	N.A.	Unguided iCBT vs. WLC[+]: ICER: £-39,753/responder iCBT generated less costs per responder.

Note:
AC Attention control group, *AD-SUS* Adult Service Use Schedule, *BDI* Beck's Depression Inventory, *BSS* Beck Scale for Suicide Ideation, *CBA* Cost-benefit analysis, *CG* control group, *CEA* Cost-effectiveness analyses, *CEAC* cost-effectiveness acceptability curve, *CES-D* Center of Epidemiologic Studies Depression Scale, *CORE-OM* Clinical Outcomes in Routine Evaluation–Outcome Measure; *CUA*, Cost-utility analyses; *CSSRI*, Client Sociodemographic and Service Receipt Inventory, *CY-BOCS* Children's Yale-brown obsessive compulsive scale, *Δ Costs* Mean incremental costs per participant (intervention group minus control group), *EQ-5D-3L/5L* European quality of life index version 5D with 3 or 5 level, *F2F* Face-to-face, *FU* Follow-up, *gCBT* Group-administered cognitive-behavioural therapy, *HAI* Health Anxiety Inventory, *iCBT* internet-based cognitive behavioural therapy, *ICD* International Classification of Diseases, *ICER* incremental cost-effectiveness ratio, *ICUR* incremental cost-utility ratio, *IG* intervention group, *iMA* internet-based interventions applying mixed approaches, *iPPI* internet-based Positive Psychology Intervention, *iPST* internet-based problem-solving therapy, *iPCT* internet-based preventive cognitive therapy, *ISI* Insomnia Severity Index, *LSAS* Liebowitz Social Anxiety Scale, *Max.* maximal probability of cost-effectiveness, *N.A.* not applicable, *N.R.* not reported; *p* probability of a treatment being cost-effective when compared with a control condition**,** *PDS-5* Posttraumatic Diagnostic Scale for DSM-5, *PSSRU* Personal Social Services Research Unit, *PDSS* Panic Disorder Severity Scale, *PHQ-9* Patient Health Questionnaire-9, *PRODISQ* PROductivity and DISease Questionnaire, *PSS-10* Perceived Stress Scale, *SC* Standard Care, *SCP* Stepped care pathway, *SE* Subjective sleep efficiency, *SF-6D* Short From six dimensions based on SF-36 Health Survey, *SPIN-17* Social Phobia Inventory, *TAU* Treatment as usual, *TAU*$^{+}$ Treatment as usual with additional information e.g. web-based psychoeducation, *TDABC* Time-driven activity-based costing method, *TiC-P* Trimbos/iMTA Questionnaire on Costs Associated with Psychiatric Illness, *uGPC* Usual General Practitioner Care, *Vs.* versus, *WHO HPQ* World Health Organization Health and Work Performance Questionnaire, *WLC* Wait list control condition, *WLC+* WLC with additional information during the waiting period, *Y-BOCS* Yale-Brown Obsessive-Compulsive Scale, *AU$* Australian Dollar, *€* Euro, £ Pound Sterling, *US$* USDollar

The significance level *$p<0.05$, ** $p<0.001$ for health-related outcomes and QALY are stated, if reported
Cost-benefit analysis was interpreted using this formula: CBA = NB>0, BCR>1, and ROI>0% = financial returns

[a] According to the location of the point estimate in the incremental cost-effectiveness plane the abbreviation South-East (SE), North-East (NE), South-West (SW), and North-West (NW), was used to indicate +/- cost and effects of the intervention compared to the control condition: SE ICERs are located in the south-east quadrant meaning that the intervention is more effective at lower costs (dominant); NE ICERs are located in the north-east quadrant meaning that the intervention is more effective at higher cost; NW ICERs are located in the north-west quadrant meaning that the intervention is less effective at higher cost (dominated); SW ICERs are located in the south-west quadrant meaning that the intervention is less effective at lower cost
[b] The WTP given certain probabilities of CE (e.g., probability of CE if the WTP is zero, *P* (WTP = 0) based on the CEAC are reported. The probabilities of the CE given certain WTP thresholds (e.g., for a WTP of £20,000, there is a 95% intervention's probability of being cost-effective, WTP ($p = 0.95$) based on the CEAC are reported.
[c] All outcomes were indexed to a 2020 reference year and converted to Pound Sterling (£) using purchasing power parities (PPPs)
[d] The χ^2 test was used for statistical analysis
[e] The independent *t*-test was used for statistical analysis
[f] The probability of the ICER given a specific WTP threshold or the WTP according to a given probability was not stated, thus the value was estimated based on the CEAC

[g] One-way ANOVAs (analysis of covariance) were used for statistical analysis

[h] A linear mixed model with restricted maximum-likelihood estimation (REML) and an unstructured (UN) covariance structure was used for statistical analysis

[i] Mixed-effects model analyses were used for statistical analysis

[j] A logistic regression model was used for statistical analysis

[k] A linear regression model was used for statistical analysis

[l] The Fisher's exact test was used for statistical analysis

[m] A bootstrapped seemingly unrelated regression equation model (bias-corrected accelerated 95% Cis) was used for statistical analysis

[n] A linear probability model considering the clustered data structure was used for statistical analysis

[Nb] The net-benefit approach was used for the analysis (net-benefit regression framework)

Supplementary table 2: Medical subject headings (MeSH) used for searching the Medline data base.

Building blocks	Treatment	Disorder	Delivery mode	Costs	Randomized trial
MEDLINE	"Psychotherapy"[mh] OR Psychotherap* OR Therap* OR Intervention OR Treatment OR Prevent* OR prevention	"Depression"[mh] OR depress* OR mood OR Bipolar OR Dysthymi* OR mania* OR anxi* OR Stress* OR Phobi* OR Panic* OR agoraphobi* OR social anxi* OR generali* anxi* OR GAD OR OCD OR "Obsessive–compulsive disorder"[mh] OR PTSD OR post-traumatic stress OR health anxiety OR hypochondria OR self-harm OR suici* OR sleep* OR insomnia	"Computer Assisted Therapy"[mh] OR Internet[mh] OR "Computer-Assisted Instruction"[mh] OR Online OR Online based OR Web OR Web based OR Internet OR Internet based OR World wide web OR Computerized OR computer	"Costs and Cost Analysis"[mh] OR "Cost-Benefit Analysis"[mh] OR "Healthcare Costs"[mh] OR "Cost of Illness"[mh] OR "Quality-Adjusted Life Years"[mh] OR Economic OR Cost utility OR Health economic OR Cost*	Efficacy OR "Randomized Controlled Trials as Topic"[mh] OR "Randomized Controlled Trial"[mh] OR Randomized controlled trial OR Randomized trial OR RCT